

Public Health Emergency Operations Center "COUSP DRC"

MVE-17 Incident Management System

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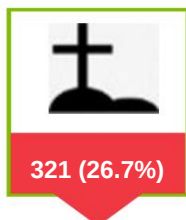
Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°042/MVB_25/06/2026

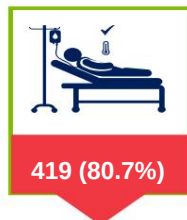
Affected provinces (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (34)	- Ituri (22/36): Aru, Aungba, Bambu, Bunia, Damas, Drodro, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia and Fataki. - North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka, Vuhovi, Mabalako and Musienene. - South Kivu (1/34): Miti-Murhesa
Reporting date	June 25, 2026
Publication date	June 26, 2026



Total confirmed cases
for the 3 provinces



Cumulative deaths
among confirmed cases
and case fatality rate



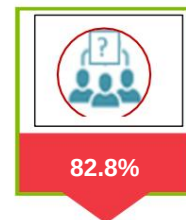
Patients in
isolation -
hospitalization



Cumulative number
of recoveries



Suspected cases of
the day



Contact tracing rates
for the 3 provinces

* Data updated following the DHIS2 database cleaning and harmonization process, including the consolidation of information from health zones, laboratories and case management structures.

Suspicious deaths occurring in the community are subject to investigation.

further investigation with a view to possible reclassification (Non-cases, probable cases)

1. HIGHLIGHTS

Forty -eight (48) new confirmed cases, including 11 deaths, were reported at the end of the day on June 25, 2026, in the provinces of Ituri (42 cases, including 7 deaths) and North Kivu (6 cases, including 4 deaths). These confirmed cases are distributed across the health zones of Rwampara (21), Bunia (9), Mongbwalu (5), and Nyankunde (2). Nizi (2), Logo (2), and Tchomia (1) in Ituri province and Beni (2), Butembo (2), and Katwa (2) in North Kivu province. Additionally, 6

deaths were recorded among Ebola patients receiving treatment at Ebola treatment centers. from the province of Ituri (3 in Mongbwalu, 1 in Bunia, 1 in Nyankunde and 1 in Bambu).

- Ten (10) patients with Ebola virus disease in Ituri have been declared cured after obtaining negative control tests.
- Joint mission of the SGI MVE-WHO-Medair surveillance team in the SOTA/ZS health area of Nyankunde to assess the needs of the local community in relation to the response to Ebola.

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For more than two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups and frequent population movements.

The population is fleeing towards Uganda, and numerous artisanal mining sites attract migrant workers. In September 2018, Ituri province had already been affected by the 10th Ebola virus (EV) outbreak, which was then raging in North Kivu. The current outbreak, caused by Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a total estimated population of nearly 15 million, with massive internal displacement and cross-border flows to neighboring countries.

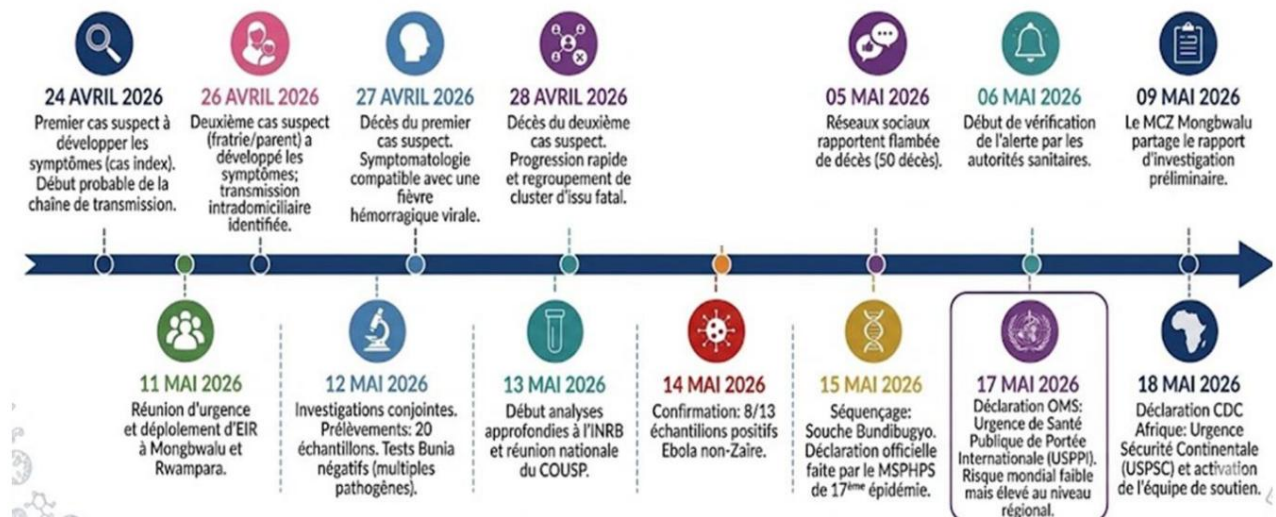


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of June 25, 2026

Table 1. Distribution of confirmed cases and deaths by affected province as of June 25, 2026.

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24 hours)
Ituri	1096	263*	24.0%	22 out of 36 (61.1%)	42
North Kivu	104	57	54.8%	11 out of 34 (32.3%)	6
South Kivu	3	1	33.3%	1 out of 34 (2.9%)	0
Total 1203 *For the province of Ituri,		321	26.7%	34 out of 104 (32.7%)	48

6 deaths were recorded among patients with Ebola virus disease in Ebola treatment centers (3 in Mongbwalu, 1 in Bunia, 1 in Nyankunde and 1 in Bambu), bringing the cumulative number of deaths among confirmed cases to 263.

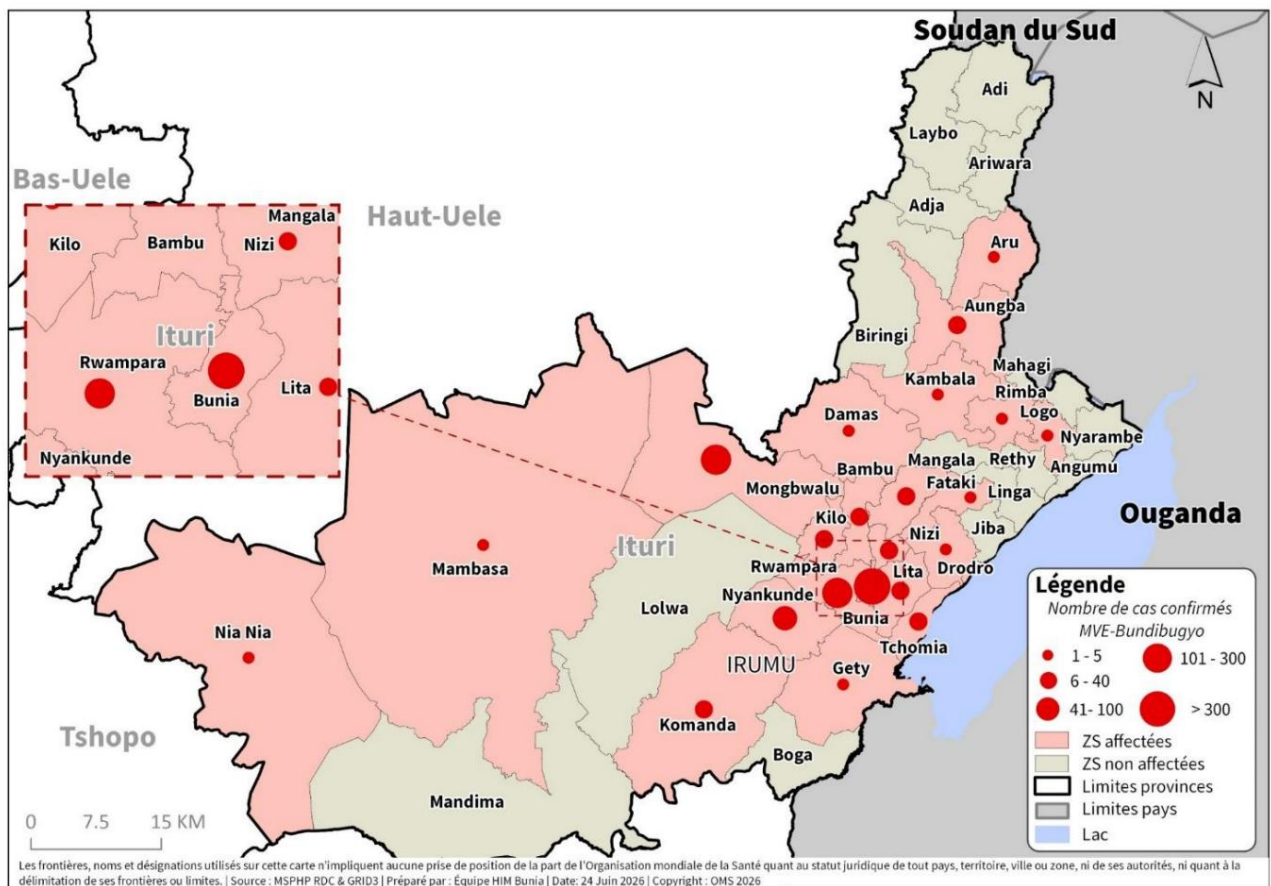
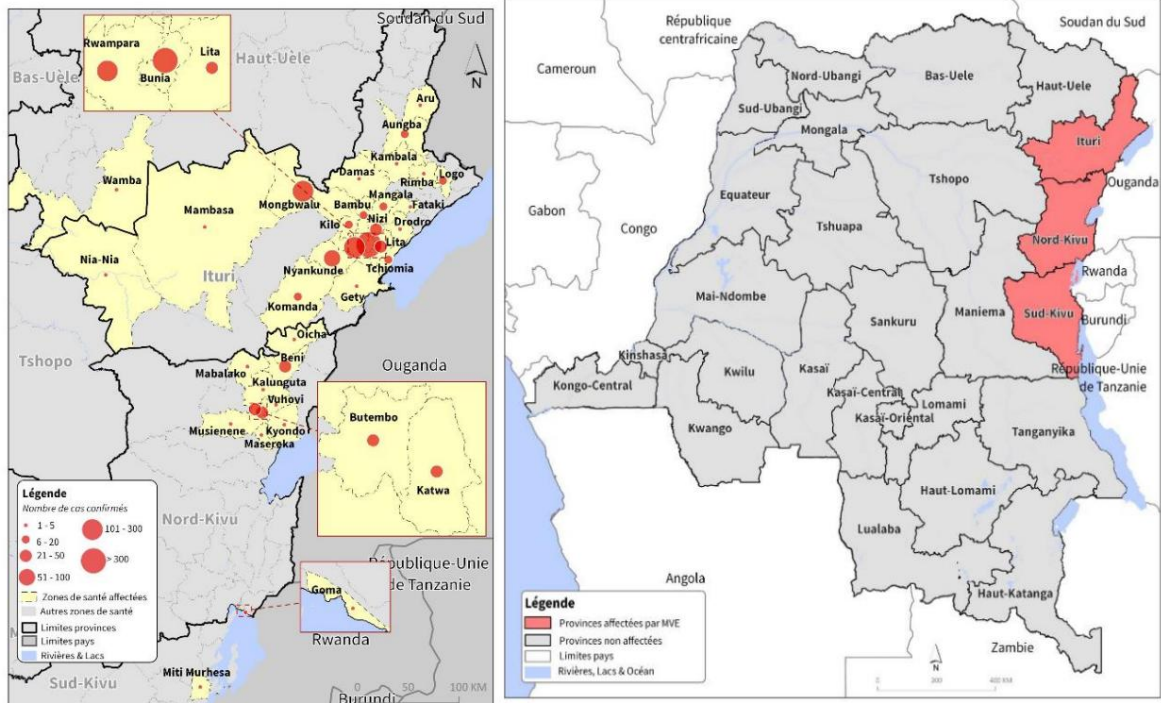
No new health zones have been affected in the last 24 hours. The number of health zones

The number of affected areas remains at 34/104 (section 3.1). Since May 26, 2026, the province of South Kivu has not recorded any confirmed cases, suggesting an absence of recent transmission.

3.2 Distribution by health zone (Cumulative total as of 25/06/2026)

- **Province of Ituri (1,096 cases):** Bunia (323), Rwampara (258), Mongbwalu (250), Nyankunde (93), Nizi (32), Lita (26), Mangala (17), Bambu (14), Komanda (13), Tchomia (10), Kilo (9), Logo (7), Aungba (6), Nia-Nia (5), Damascus (4), Rimba (3), Aru (3), Mambasa (2), Drodoro (1), Gety (1), Kambala (1), and Fataki (1). Seventeen (17) additional cases are awaiting precise breakdown by health zone in the province.
- **North Kivu Province (104 cases):** Katwa (35), Butembo (33), Beni (22), Oicha (3), Kyondo (3), Kalunguta (2), Musienene (2), Goma (1), Masereka (1), Vuhovi (1) and Mabalako (1).
- **South Kivu Province (3 cases):** Miti-Murhesa (3).

3.3 Epidemic mapping



Figures 2, 3 and 4. Spatial distribution of confirmed Ebola cases by affected health zones and in the three affected provinces as of June 25, 2026.

Table 2. Distribution of confirmed cases and deaths of Ebola virus disease by province and health zone (June 25, 2026)

Province / Health Zone	Cumulative confirmed cases (n)	Cumulative nonconfirmed deaths (n)	Case fatality rate (CFR, %)
ITURI	1096	263	24.0%
Bunia	323	64	19.8%
Rwampara	258	43	16.7%
Mongbwalu	250	109	43.6%
Nyankunde	93	11	11.8%
Nizi	32	6	18.8%
Lita	26	6	23.1%
Komanda	13	3	23.1%
Bamboo	14	3	21.4%
Kilo	9		11.1%
Mangala	17	1	47.1%
Tchomia	10	8	0.0%
Damascus	4	0	0.0%
Aungba	6	0	33.3%
Rimba	3	2	0.0%
Aru	3		33.3%
Mambasa	2		50.0%
Logo	7	0	0.0%
Drodoro			100.0%
Gety		1	0.0%
Kambala			100.0%
Nia-Nia	1	1	60.0%
Fataki	1 1 5 1	0 1 0 1 3 0	0.0%
Other areas not yet identified	17	0	0.0%
NORTH KIVU	104	57	54.8%
Butembo	33	14	42.4%
Katwa	35	22	62.9%
Blessed	22	13	59.1%
Oicha	3	2	66.7%
Kalunguta	2		50.0%
Kyondo	3	1	66.7%
Goma		2	0.0%
Masereka		0	0.0%
Vuhovi	1 1 1	0 1	100.0%
Mabalako		0	0.0%
Musienene	1	2	100.0%
SOUTH KIVU	23	1	33.3%
Miti-Murhesa	3	1	33.3%
TOTAL 321	1203		26.7%

In the Democratic Republic of Congo, the epidemiological situation of the 17th epidemic of Viral Disease

Ebola (EVD) continues to spread, **with 48 new confirmed cases, including 11 deaths**, recorded during the past few days.

In the last 24 hours, it was also noted that **6 patients** with Ebola virus disease (previously confirmed cases) being monitored in various facilities have died. Thus, the cumulative total stands at **1,203 confirmed cases** and **321 deaths**, representing an overall case fatality rate of **26.7%**.

Provincial dynamics and case concentration

The **Ituri** province remains the absolute epicenter of the epidemic, accounting for **91.1%** of cases (n=1,096) and **81.9%** of deaths (n=263), representing a case fatality rate of **24.0%**. The health zones of Bunia (323 cases), Rwampara (258 cases), and Mongbwalu (250 cases) concentrate slightly more than three-quarters of all reported cases in this province, making them the most affected. Furthermore, moderate case fatality rates were observed in the health zones of Bunia (19.8%) and Rwampara (16.7%), while the **Mongbwalu** zone exhibited marked severity.

With a case fatality rate of **43.6%** (109 deaths out of 250 cases), this suggests ongoing challenges in early intervention and access to care. It should be noted that 17 additional cases (with no associated deaths) have been reported, while the health zones of origin remain to be identified, which could indicate a geographical spread requiring further investigation.

Severity of the epidemic in North Kivu

Since the start of the epidemic, **104 confirmed cases** and **57 deaths** have been reported in North Kivu province, representing a **case fatality rate of 54.8 %**. This province presents the most concerning profile in terms of severity, which is higher than the national average. This high case fatality rate is concentrated in the health zones of **Katwa** (62.9%), **Beni** (59.1%), and **Oicha** (66.7%). The Katwa health zone has the highest number of reported cases (n=35), followed by the Butembo health zone (n=33) with a lower case fatality rate (42.4%). A case fatality rate of 100% was observed in the exceptionally small Vuhovi and Musienene health zones, highlighting the danger posed by the virus in this region and the need for an immediate clinical response.

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

National / Ituri:

- Supervision of field teams and the various patient care structures.
- Holding the coordination meeting and supporting the pillars of the response.

4.2 Epidemiological surveillance

Table 3. Management of epidemiological alerts (24h) as of June 25, 2026

Table of management of epidemiological alerts (24h) as of June 23, 2023					
Indicator		Ituri	North Kivu	South Kivu	Total
Report (D-1)		13	0	1	14
New Alerts		410	740	12	1162
Total alerts for the day		423	740	13	1176
Alerts investigated		423	740	12	1175
Investigation rate		100.0%	100.0%	92.3%	99.9%
Alerts confirmed	Alive	135	49	4	188
	Deceased	38	39	0	77
Total suspected cases for today		173	88	4	265

During the course of June 25, 2026, **1,176 alerts** were reported across the three provinces, **1,175 (99.9%)** were investigated, **265 suspected cases including 77 deaths** were identified (Table 3).

Table 4. Contact tracing of confirmed cases as of June 25, 2026

Province	Contacts under follow-up (n)	Contacts viewed (n)	Follow-up rate (%)
Ituri	7092	5,861	82.6%
North Kivu	2184	1,821	83.4%
South Kivu		18	100.0%
Total	189294	7,700	82.8%

- Continued in-depth investigations and exhaustive listing and systematic follow-up of contacts around confirmed cases in the ZS (narratives of confirmed cases, number of contacts listed and followed per case).
- **774 new contacts** were identified, including 596 in 7 health zones in Ituri province and 178 in 3 health zones in North Kivu province. In addition, 2 **contacts successfully completed their 21-day follow-up in the Bunia health zone (Ituri province)**.

Update on checkpoint and point of entry (PoC/PoE) activities

- Installation of handwashing facilities at PoC Mahagi, Uguro and Nioka.
- Activation of the Mabakese/Mambasa PoC.
- Provision of high-visibility vests to service providers at the Mandje and Nyamusasi PoCs • **In**

Ituri : One (1) alert was notified to the Foner Mambasa PoC; the alert was invalidated after testing secondary.

- **In North Kivu** : 5 alerts were notified, of which 1 was validated, isolated and referred to the CT.

Table 5. Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC), as of June 25, 2026

INDICATORS	Ituri	North Kivu	Overall
Proportion of PoC enabled and PoE enhanced	89.0% (49/55)	92.3% (12/13)	89.7% (61/68)
Number of people who went through PoE/PoC testing,	91,403	51,100	142,503
% of travelers screened, % of	95.4%	97.7%	96.2%
travelers who washed their hands, % of	94.9%	97.7%	95.9%
travelers made aware of the issue	97.1%	97.7%	97.3%
Number of alerts notified	1	5	6
Number of validated live alerts	0	1	1
Number of lifeless bodies intercepted today	0	0	0
Number of problematic contacts intercepted today; %	0	0	0
of validated active alerts referred to CT/CTE	0.0%	100.0%	100.0%
% of the lifeless bodies swabbed	0.0%	0.0%	0.0%
Number of positive results of referred suspected cases	0	0	0

4.3. Laboratory

- **Ituri** : 165 samples collected and analyzed (positivity 25.4%; n=42).
- **North Kivu** : 95 samples received and analyzed, **6 came back positive, i.e. 6.3%**.
- **South Kivu** : 4 samples were collected and analyzed, all came back negative.

4.4. Infection Prevention and Control (IPC)

Ituri: 38/45 safe and dignified burials (SDBs) were carried out, with 7 postponements; 17 community swabs were performed; 5 scorecard assessments were carried out in the ESSs, and 13 ESSs were monitored.

accompanied; decontamination of 3 ESS, 20 households around confirmed cases decontaminated, 38 means of transport decontaminated; 191 PPL trained and 55 Recos/community leaders briefed on standard precautions.

North Kivu : Provision of PCI M1 and M2 kits to the 18 priority ESS (10 in the Nyiragongo health zone and 8 in the Karisimbi health zone); provision of PCI inputs (PPE, chlorine, OMO detergent, buckets, boots, household gloves, aprons, etc.) to the community PCI teams in the Karisimbi and Nyiragongo health zones; preparation of the joint WHO-DPS-INRB and multidisciplinary SURVEPI-LABO-PCI-PECH-CREC mission in the health zones of Kayna, Alimbongo, Rutshuru and Rwanguba to strengthen zonal coordination as well as the various sub-committees put in place; support of priority health establishments in the hotspot health zones to raise the level of IPC after the scorecard assessments carried out, with briefing of 106 health providers on the different themes of IPC in the context of the Ebola epidemic.

South Kivu : Distribution of IPC and PPE inputs in 4 ESS of the Katana Health Zone with support from WHO; briefings of the 8 IPC supervisors including 2 from the DPS, 3 from the Miti-Murhesa Health Zone and 3 from the Katana Health Zone; decontamination of 4 households of suspected cases including 3 in the Katana Health Zone and 1 in the Miti-Murhesa Health Zone; bio-cleaning of the 10 schools selected as state examination centers.

4.5 Holistic care

- **Ten (10) patients** were declared cured in Ituri, after obtaining negative control results.

Thus, the cumulative number of recovered patients is **148, including 139 in Ituri, 7 in North Kivu and 2 in South Kivu.**

- Continued medical care, transfers to Ebola Treatment Centers (ETCs), notification of results, and nutritional monitoring of patients. A total of 706 hot meals were served, including 111 to confirmed cases, 123 to suspected cases, 437 to people with HIV/AIDS (PPIs), and 35 to caregivers.
- Twenty-seven (27) patient households received food aid.

Table 6. Patient movement within healthcare facilities (CTE/CT/CI) as of June 25, 2026

Indicator	Ituri	North Kivu	South Kivu	Together
Patients in bed (Day -1)	313	56	13	382
Total admissions (24h)	60	12	4	76
Exits (24h)				
Deceased (Suspects/Confessions)	7	0	0	7
No case	21	0	0	21
Cured	10	0	0	10
Escapees (Suspects/Confessions)	1	0	0	1
Transferred to the HGR	0	0	0	0
Total outflows	39	0	0	39
Patients in isolation (End of Day),	334	68	17	419
including confirmed cases	193	27	0	220
(NC+AC) and suspected cases	141	41	17	199
Overall occupancy rate	89.8%	66.0%	38.6%	80.7%

Mental health activities and psychosocial support

Table 7. Key indicators of mental health and psychosocial support activities as of June 25, 2026

KEY INDICATORS	ITURI	NORTH KIVU	SOUTH KIVU
New confirmed cases admitted to the CTE who benefited from SMSPS interventions	4 (57.1%)	0	0
Confirmed cases currently being monitored that have received interventions SMSPS	53 (39.8%)	6 (100.0%)	0
New suspected cases that benefited from SMSPS interventions	1 (3.1%)	7 (100.0%)	0
Suspected cases under follow-up that have received interventions SMSPS	67 (73.6%)	15 (100.0%)	1 (100.0%)
Laboratory results have been announced.	10	17	3
Including positive ones	4	1	0
Number of children separated in daycare who benefited from interventions SMSPS	0	0	0
Number of children separated in the community who benefited from SMSPS interventions	0	0	11
Number of accompanying persons at the CTE who benefited from interventions SMSPS	111	9	6
Number of PPLs who benefited from SMSPS interventions	9	7	15
Number of household and community members who benefited from SMSPS interventions	51	34	0
Number of EDS supported	0	2	0

4.6 Continuity of care

- Continued work on extending CTEs, notably Rwampara and HGR/Bunia; progressive improvement of the technical platform; transfer of patients from ESS/communities to the sites selected for care; briefing on data reporting to the Elikya CTE.

4.7 Risk communication and community engagement

Ituri: Training of national and provincial trainers in community-based communication and surveillance; 3 community dialogues organized on Ebola virus disease (EVD) preventive measures, community-based surveillance, and care for 125 people (60 men and 65 women); 4,221 people reached during 64 mass awareness campaigns on compliance with EVD preventive measures, care, community alerts and EDS, reporting of animal disease alerts, etc.; 7,718 people reached (3,239 men and 4,479 women) in households visited by 470 community health workers in the health zones of Mongbwalu, Rwampara, and Nyankunde; dissemination of 18 public service announcements and messages on EVD preventive measures; hosting of two radio programs on EVD preventive measures in the health zones of Mongbwalu and Nia-Nia; hosting of a radio program on the RCRN (Regional Coordination of National Respiratory Diseases).

Support for the pillars of the response : Surveillance: Facilitate contact tracing around confirmed cases and identification of referrals for contact tracing in the Hoho health district (Rwampara health zone), and contact tracing in the Rwampara, Dele, Tchungu, and Hoho health districts. **IPC :** Prepare close contacts to collaborate in decontamination activities.

North Kivu: 1,925 community health workers mobilized, 34,569 people (19,602 women and 14,967 men) reached through 11,471 households visited in the health zones of Goma, Karisimbi, and Nyiragongo; 316 community leaders

involved in the Ebola response in the health zones of Goma, Karisimbi, Kirotshé and Nyiragongo; 3,705 people reached - including 1,190 in public places (863 via Mobile Clinic/CREC-IMC in Kirotshé, 327 via U-Report); 2,515 via community talks - 18 local radio stations engaged, 3 formats produced and broadcast; 2,315 finalists sensitized on the last day of the State exams on hand hygiene; only 1 rumor collected and clarified for 69 community alerts reported; 1 Ebola survivor and her family committed to testifying and contributing to the response - a major strategic result for community trust and the fight against stigma.

South Kivu : 1,305 households visited, 7,172 people sensitized, 12 radios broadcast the messages, 54 leaders engaged; awareness-raising of final-year students at the Kavumu state examination center; 345 people reached by the prevention message; educational talk with 28 students from ITM Katana on Ebola prevention measures and signs; 12 radio stations broadcast the Ebola prevention message in 10 health zones of the province.

4.8. Logistics

Ituri

- In total, 24 vehicles and 4 ambulances were made available to the various pillars to ensure the mobility.
- Installation of electronic chips on 100 motorcycles provided by UNICEF and 60 government motorcycles central.
- Delivery of PCI inputs for EDS and ongoing preparation of PCI inputs for CSR Sota.
- Continuation of development work at the HGR/Bunia and HGR/Rwampara CTEs.
- Operationality of the CTEs HGR/Bunia, HGR/Nyankunde, ISTM/Nyankunde, HGR/Aru, HGR/Rwampara and CME/Bunia.

4.9. Security

- The security situation remains calm but unpredictable in Bunia and surrounding areas.

4.10. PSEA (Prevention of Sexual Exploitation and Abuse)

- Continued briefings for stakeholders, community awareness-raising on exploitation, abuse and sexual harassment, as well as the signing of codes of good conduct.
- **South Kivu :** Briefing for journalists on risk communication regarding Ebola Virus Disease (EVD) and AIDS at the South Kivu Provincial Health Directorate (DPS), with financial support from UNICEF, implemented by REMED with technical support from WHO. A total of 40 participants, including 15 women, took part.



5. MAIN CHALLENGES, IMPACTS AND REQUIRED ACTIONS

Challenge	Impact	Actions required
No. 1 Reluctance to undergo post-mortem sampling (EDS/swabs) and community resistance	Confirmation delays, underestimation of cases, disruption of key activities	Intensify CREC, involve religious and community leaders, and strengthen team security.
2 Insufficient capacity of the CTEs and close to saturation in Ituri (89.8%)	Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTE, deploy additional temporary structures
3 Poor performance in contact tracing (82.8% vs target 95%)	Uninterrupted transmission chains	Train, deploy and motivate community liaisons (RECOs), enhanced supervision
4 Low reporting of alerts in some affected areas	Undetected transmission, silent spread of the epidemic	Strengthen community surveillance, improve the reporting and rapid investigation of alerts
5 confirmed cases not yet broken down by health zone (17 case)	Major gaps in the reconstruction of transmission chains	Conducting a data reconciliation, Remove any potential duplicates, and reclassify or clarify the health zone and status (deceased, recovered, active, etc.). Active verification in the field.
6 Insufficient supply of MEG and medical supplies	Weakening of overall care	Advocacy and urgent supply of essential medicines
7 Insufficient PCI inputs (PPE, chlorine) and limited training	High risk of nosocomial infections (78 cases) HCW (including 18 deaths, case fatality rate 23.1%)	Urgent supply, strengthening of IPC training
8 Stockpile of unanalyzed samples (backlog) at the laboratory in North Kivu and delayed diagnosis	Delayed confirmation, high lethality (54.8%)	Deploy a laboratory catch-up plan, strengthen diagnostic capabilities
9 Insufficient number of ambulances and isolation facilities (gap of 20 centers)	Transfer delays and isolation increase the risk of transmission	Urgent deployment of additional logistical resources and infrastructure
10. Insufficient coordination and low utilization of COUSP frameworks	Operational inefficiency and delayed decisions	Strengthen coordination mechanisms at all levels, make operational the Provincial COUSP
11. Insufficient resources financial (gap of 20 million USD)	Limitation of the implementation of all pillars of the response	Urgent mobilization of resources, advocacy with partners
12 Insecurity and limited access (CODECO, ADF)	Restrictions on operations, limited access to high-risk areas	Strengthening security coordination and humanitarian access
13 Continuity of essential services (CEHS) compromised	Increase in indirect mortality (not Ebola)	Effective implementation of free healthcare, strengthening of CEHS
14. High population mobility	High risk of rapid geographic expansion	Strengthening surveillance at PoE/PoC sites and cross-border coordination

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With technical support from the World Health Organization (WHO) and
of Africa CDC